

Name : Mr. ROHIT  
Lab No. : 463577601  
Ref By : DR ADEEB SINGH  
Collected : 9/12/2024 5:10:00PM  
A/c Status : P  
Collected at : Hexamed Healthcare Services LLP  
25-270/6, Taluka Bahu, Main Bypass Road, NH1A,  
Channi Himmat,  
Jammu,

Age : 16 Years  
Gender : Male  
Reported : 12/12/2024 1:32:00PM  
Report Status : Final

### Test Report

#### STONE ANALYSIS BY FTIR



Sample: Kidney Stones

Aggregate Weight: 3.131 gm

Composition : Calcium Oxalate Monohydrate 80% Ammonium Hydrogen Urate 20%

(Representative stone analysed)

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|                     |                                                                                                                 |                      |                        |
|---------------------|-----------------------------------------------------------------------------------------------------------------|----------------------|------------------------|
| <b>Name</b>         | : Mr. ROHIT                                                                                                     | <b>Age</b>           | : 16 Years             |
| <b>Lab No.</b>      | : 463577601                                                                                                     | <b>Gender</b>        | : Male                 |
| <b>Ref By</b>       | : DR ADEEB SINGH                                                                                                | <b>Reported</b>      | : 12/12/2024 1:32:00PM |
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| <b>A/c Status</b>   | : P                                                                                                             |                      |                        |
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### Test Report

#### (Representative stone analysed)

Kidney stone (Nephrolithiasis) develops when urine becomes supersaturated with crystal forming substances such as Calcium, uric acid, oxalate etc.(resulting from dehydration or genetic predisposition) or due to decrease in urinary concentration of substances such as magnesium, citrate etc. that inhibit stone formation. Kidney stones tend to recur; approximately 50% of patients who form one stone form another in 10 years. Nephrolithiasis is considered to be a disease of affluence because it is more prevalent in conditions like Obesity, Hypertension & Diabetes. For patients with multiple kidney stones 24-hour urine chemistries should be measured to identify abnormalities that predispose to kidney stones, which guides dietary and pharmacologic therapy to prevent future stone events.

| STONE TYPE                              | FREQUENCY (%) | CAUSES                                                                                                                                                                                                                                                                                                                                       |
|-----------------------------------------|---------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Calcium oxalate / mix                   | 75            | <ul style="list-style-type: none"> <li>Hypercalciuria with Hypercalcemia (Hyperparathyroidism, Vitamin D excess, malignancy)</li> <li>Idiopathic hypercalciuria</li> <li>Hypocitraturia</li> <li>Alkaline urine, low urine volume</li> <li>High dose of vitamin C</li> <li>Metabolic syndrome</li> <li>Inflammatory bowel disease</li> </ul> |
| Calcium Phosphate                       | 5             | <ul style="list-style-type: none"> <li>Distal Renal Tubular acidosis</li> <li>Primary Hyperparathyroidism</li> <li>Chronic urinary tract infection</li> <li>Hypercalciuria and/or Hypophosphaturia</li> </ul>                                                                                                                                |
| Magnesium Ammonium Phosphate (Struvite) | 10-20         | Recurrent urinary tract infection                                                                                                                                                                                                                                                                                                            |
| Uric Acid                               | 5-10          | <ul style="list-style-type: none"> <li>Hyperuricosuria and /hyperuricosemia</li> <li>Metabolic syndrome (Diabetes,obesity, Hypertension)</li> <li>Acidic urine (pH&lt;5.5)</li> <li>Gout</li> </ul>                                                                                                                                          |
| Cystine                                 | 1             | Cystinuria (A genetic disorder)                                                                                                                                                                                                                                                                                                              |

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Page 5/14



|                     |                                                                                                                 |                      |                        |
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| <b>Name</b>         | : Mr. ROHIT                                                                                                     | <b>Age</b>           | : 16 Years             |
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### Test Report

### DIETARY RECOMMENDATIONS FOR PREVENTION OF RECURRENCE

#### General Dietary measures:

- Fluid intake should be between 2.5-3L/day or it should be sufficient to produce urine output of 2L/day
- Diet rich in fibres, fruits (orange, lemon, apple etc) & vegetables
- Maintain adequate calcium intake (1000-1200 mg/day)
- Aim to achieve/maintain healthy weight
- Reduce intake of animal protein (meat & dairy)

#### Specific measures based on metabolic evaluation and stone composition:

| STONE COMPONENT                                             | METABOLIC PROFILE               | SUGGESTED DIETARY RECOMMENDATION                                                                                                                                                           |
|-------------------------------------------------------------|---------------------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Calcium oxalate or combined calcium oxalate/phosphate stone | Hypercalciuria                  | Salt restriction                                                                                                                                                                           |
|                                                             | Hyperoxaluria                   | Calcium supplements with meal<br>Restrict items rich in oxalate eg.<br>Soya protein, Spinach, Nuts,<br>Strawberries, Cocoa, Chocolates,                                                    |
|                                                             |                                 | beetroot, carrot, radish etc                                                                                                                                                               |
|                                                             | Hyperuricosuria                 | <ul style="list-style-type: none"> <li>• Limit animal protein &amp; purine intake (Fish, lamb, beef, chicken, pork etc), cauliflower, Mushroom</li> <li>• Reduce alcohol intake</li> </ul> |
|                                                             | Hypocitraturia                  | Increase citrate rich food (Orange, lemon, apple etc)                                                                                                                                      |
| Uric Acid                                                   | Hyperuricosuria & Hyperuricemia | <ul style="list-style-type: none"> <li>• Limit animal protein &amp; purine intake (Fish, lamb, beef, chicken, pork etc), cauliflower, Mushroom</li> <li>• Reduce alcohol intake</li> </ul> |
|                                                             | Metabolic syndrome              | Lifestyle changes, weight loss, Diabetes control                                                                                                                                           |
| Cystine                                                     |                                 | <ul style="list-style-type: none"> <li>• Increase fluid intake to 3-4L, Urine output should at least be 3L</li> <li>• Restrict sodium &amp; protein intake</li> </ul>                      |

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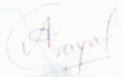




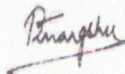
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### Test Report

  
DMC - 67327

Dr Anjalika Goyal  
MD, Biochemistry  
Consultant Biochemist

  
DMC - 89619

Dr Himangshu Mazumdar  
MD, Biochemistry  
Sr. Consultant Biochemist

  
DMC - 9550

Dr Nimmi Kansal  
MD, Biochemistry  
Technical Director - Clinical Chemistry  
& Biochemical Genetics

### Test Report

#### IMPORTANT INSTRUCTIONS

• Test results released pertain to the specimen submitted. • All test results are dependent on the quality of the sample received by the Laboratory.  
• Laboratory investigations are only a tool to facilitate in arriving at a diagnosis and should be clinically correlated by the Referring Physician. • Report delivery may be delayed due to unforeseen circumstances. Inconvenience is regretted. • Certain tests may require further testing at additional cost for derivation of exact value. Kindly submit request within 72 hours post reporting. • Test results may show interlaboratory variations. • The Courts/Forum at Delhi shall have exclusive jurisdiction in all disputes/claims concerning the test(s) & or results of test(s). • Test results are not valid for medico legal purposes. • This is computer generated medical diagnostic report that has been validated by Authorized Medical Practitioner/Doctor. • The report does not need physical signature.

-----End of report-----

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